

Fictive Chat

Hypertension — fictive patient chats

Illustrative dialogues between a lay patient and a health assistant. Derived from the patient scenarios and message-library tone in the WHO *Be He@lthy Be Mobile — mHypertension handbook* (2020). All names and specifics are invented. Numbers are illustrative; clinical decisions must come from a qualified health worker.

These transcripts are intended as retrieval examples — they show the chatbot what a good answer looks like for common patient situations: plain language, benefit-framing, no clinician jargon, source-cited where useful.

Chat 1 — Newly diagnosed, scared

Fatou: I went for a check-up yesterday and the nurse said my blood pressure is 158/95. She gave me pills. I don't feel sick. Am I going to have a stroke?

A reading like that means you have high blood pressure, but it does not mean a stroke is about to happen today. What it does mean is that, if nothing changes, your risk over the next few years is higher than it should be. The good news: the pills she gave you and a few changes at home can bring those numbers down within weeks.

Fatou: I really don't feel anything. Do I have to take the pills?

This is the hardest part for almost everyone with high blood pressure — it is silent. You won't feel the pills working, just like you don't feel the pressure being too high. Take them every day at the same time, exactly as prescribed. If you feel any side effects in the first weeks, don't stop — call the clinic and they will adjust the medicine.

Fatou: What should I change at home?

Three things make the biggest difference:

1. **Less salt.** Aim for less than one teaspoon a day across everything you eat. Stock cubes and bouillon are the hidden culprits.
2. **More fruit and vegetables.** Five portions a day.
3. **Walking.** Try thirty minutes most days. It can be split into two walks if that is easier.

Would you like me to send you a daily reminder to take your medicine?

Chat 2 — Asking about food

Lamin: I love jollof rice with the Maggi cube. Do I have to give it up?

You don't have to give up jollof — you just need to bring the salt down. A Maggi cube has about as much salt as your whole daily allowance, so even one cube in a pot for the family is already over the limit. Try:

- Half a cube instead of a whole one, and add it later in cooking
- More onion, garlic, ginger, scotch bonnet, thyme and bay leaf for the flavour

- Squeeze of lemon at the end

Your taste buds adapt within a couple of weeks — what tastes “bland” now will taste normal soon.

Lamin: What about bread for breakfast?

Bread can be surprisingly salty. If you eat it every day, that adds up. Cutting back even to four mornings a week instead of seven, and topping it with avocado or egg instead of butter and processed meat, will help.

Chat 3 — Side effects

Aminata: I started the pills two weeks ago and I have a dry cough that won't go away. Is that from the medicine?

A dry cough is a common side effect of one specific family of blood pressure medicines (the ACE inhibitors — names ending in -pril). It is not dangerous but it is annoying, and there is an alternative your health worker can switch you to that doesn't cause the cough.

Please don't stop the medicine on your own. Call the clinic, tell them about the cough, and ask if you can switch. The blood pressure control matters more than which family of pill you are on.

Chat 4 — Missed doses, weekend traveller

Modou: I went to the village for the weekend and forgot my pills. Will I be okay?

A couple of days off your medication isn't usually an emergency, but your blood pressure will start to creep back up by the third or fourth day. Take your regular dose as soon as you get home — don't take a double dose to “catch up.”

For next time: - Keep a spare strip of pills in your bag for unplanned trips. - Take a photo of your prescription so a pharmacy can re-supply you anywhere in the country. - If you travel often, ask your health worker for a three-month prescription instead of monthly.

Chat 5 — Red-flag triage

Ousman: My head is hurting really bad, I can't see properly, and I feel like I'm going to be sick. I checked my pressure and it's 195/115.

Stop, this is serious. Please go to the hospital now. Don't drive yourself — call a family member, a neighbour or a taxi to take you, or call the emergency number.

While you are waiting: - Sit down somewhere quiet, don't lie flat. - Don't take an extra dose of your medication unless your health worker has specifically told you to. - Bring your medication list with you so the hospital knows what you are on.

The combination of a very high reading and those symptoms (severe headache, vision changes, nausea) is what we call a hypertensive emergency, and it needs to be treated by a doctor today, not tomorrow.

Chat 6 — Exercise, starting from zero

Binta: My nurse said I should exercise but I haven't done any exercise since school and I'm 56. I don't even know where to start.

You don't need to start with anything intense. Walking is the most underrated medicine for blood pressure, and it costs nothing.

Try this for the first week: - **Day 1-3:** A 10-minute walk after dinner. Just out and back. - **Day 4-7:** Same walk, plus one more 10-minute walk in the morning or at lunch.

That's already 20 minutes a day, five days a week. Once that feels normal — usually within two weeks — extend one of the walks to 20 minutes. Goal is 30 minutes a day, most days. Pace should be brisk enough that you breathe faster but can still talk. No need to run.

Binta: What if my knees hurt?

Then walk on flatter ground, wear shoes with cushioning, and try shorter more frequent walks. Swimming and cycling are also kind to the knees if you have access. Don't force through pain — tell your health worker if it persists.

Chat 7 — Family member asking

Mariama: My husband was just diagnosed with high blood pressure. What can I do to help him?

The single most useful thing is to make the changes household-wide rather than as a “diet” just for him:

- **Cook with less salt and fewer stock cubes for everyone.** Nobody needs as much salt as we typically use, and it stops feeling like he is being singled out.
- **Buy more fruit and vegetables for the home.**
- **Walk together** in the evening. Twenty minutes after dinner is also good for sleep.
- **Help him remember the pills** — put them by the toothbrush or next to his morning tea, so they get taken at the same time each day.

You might also want to have your own blood pressure checked, since high blood pressure often runs in families and in households that share the same diet.